

1. Problem

Emergency room (ER) utilization remains a major driver of avoidable healthcare spending.

Our analysis shows:

- ER utilization rate: **208 visits per 1,000 members**
- **40.18%** of ER visits are classified as avoidable
- Average ER visit cost: **€3,400**
- PCP engagement rate: **48.45%**

Low primary care engagement combined with high avoidable ER utilization suggests structural gaps in care coordination.

This creates three critical issues:

- Excess medical spends
- Strained emergency capacity
- Suboptimal patient care pathways

Without intervention, avoidable ER utilization will continue to inflate total cost of care and reduce system efficiency.

2. Key Findings

- **High Avoidable ER Utilization**

40.18% of ER visits are non-emergent and potentially redirectable to primary or urgent care settings.

- **Cost Concentration Among High-Utilization Providers**

Providers with above-average ER rates also show elevated average cost per visit, creating compounded financial impact.

- **Low Primary Care Engagement**

Only 48.45% of members demonstrate recent PCP interaction, indicating limited preventative care touchpoints.

- **Cost Sensitivity to Utilization Reduction**

Even modest reductions in avoidable ER visits produce measurable cost savings due to the €3,400 average cost per visit.

- **Variation Across Risk Segments**

Lower-risk populations still exhibit avoidable ER utilization, suggesting behavioural and access drivers beyond clinical severity.

3. Financial Impact

Based on current utilization levels:

- ER utilization: **208 visits per 1,000 members**
- Avoidable rate: **40.18%**
- Average ER visit cost: **€3,400**

This indicates a significant portion of emergency spend is preventable.

Estimated Annual Savings Potential

If avoidable ER utilization is reduced:

- **10% reduction:** ~4% total ER cost savings
- **20% reduction:** ~8% total ER cost savings
- **30% reduction:** ~12% total ER cost savings

Given the current cost structure, even a moderate 20% reduction in avoidable ER visits represents meaningful financial impact without structural system overhaul.

These savings can be redirected toward:

- Primary care expansion
- Preventive programs
- Care coordination initiatives

Avoidable ER reduction represents one of the highest-leverage cost optimization opportunities in the current utilization model.

4. Recommended Actions

1. Increase Primary Care Engagement

Expand proactive outreach to members with low PCP utilization.

Target members with prior avoidable ER visits and limited recent primary care interaction.

Focus areas:

- Appointment reminders
- Same-day PCP access
- Extended clinic hours
- Virtual primary care options

Objective: Shift non-emergent utilization toward lower-cost settings.

2. Target High-Variation Providers

Identify providers with above-average ER rates and cost per visit.

Implement:

- Performance benchmarking
- Referral pattern review
- Care coordination support

Objective: Reduce unnecessary ER referrals and improve outpatient management pathways.

3. Risk-Segmented Intervention Strategy

Prioritize interventions for:

- Members with prior ER utilization
- Moderate-risk populations with low PCP engagement
- Behavioural health-linked ER patterns

Objective: Concentrate resources where cost reduction probability is highest.

4. Behavioural & Access Interventions

Address non-clinical drivers of ER utilization:

- After-hours access gaps
- Transportation barriers
- Health literacy challenges

Objective: Reduce ER visits driven by convenience or access limitations.

5. Measurement Plan

To ensure accountability and measurable impact, the following metrics should be tracked quarterly:

Primary Metrics

- ER visits per 1,000 members
- Avoidable ER percentage
- Average ER visit cost
- PCP engagement rate

Impact Metrics

- Reduction in avoidable ER visits (absolute and %)
- Total ER cost trend
- Cost per member per month (PMPM)
- High-utilization member count

Provider-Level Monitoring

- ER rate by provider
- Cost per ER visit by provider
- Variance from network average

Reporting Cadence

- Quarterly executive review

- Monthly operational dashboard tracking
- Annual financial impact assessment

Success should be defined not only by reduced ER utilization, but by sustained improvements in care coordination and cost stability.

6. Risks & Mitigation

1. Access Constraints

Risk: Limited primary care capacity may restrict redirection of ER visits.

Mitigation:

- Expand telehealth availability
- Introduce extended hours
- Partner with urgent care networks

2. Member Behaviour Persistence

Risk: Habitual ER use may persist despite outreach.

Mitigation:

- Targeted education campaigns
- Incentive-based engagement models
- Personalized care navigation support

3. Provider Resistance

Risk: Providers may resist performance benchmarking.

Mitigation:

- Transparent performance reporting
- Collaborative improvement planning
- Shared savings alignment

4. Short-Term Cost Shift

Risk: Temporary increase in outpatient utilization may precede ER reduction.

Mitigation:

- Monitor total cost of care, not isolated ER metrics
- Focus on net financial impact